PEDIATRIC NURSING (THEORY) · LECTURE 10 · QUESTION BANK

Rheumatic Fever

الحمى الروماتيزمية — بنك الأسئلة

46	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

أسئلة سابقة · ميدترم 2024 (ترم أول 2023-2024) · ميدترم 2022

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 ميدترم 2022

Which of the following nursing diagnoses is most appropriate for a child with rheumatic fever?

a. High risk for injury
b. Imbalanced nutrition: more than requirements
c. Delayed growth and development
d. Impaired gas exchange

2 ميدترم 2022

A nurse is preparing a child for discharge after being diagnosed with rheumatic fever. Which is the correct instruction?

a. Only give penicillin for dental work
b. Give aspirin for management of chorea
c. Give penicillin until 21 years old
d. Give penicillin for one month total

3 ميدترم 2024 (ترم أول 2023-2024)

Which of the following instructions would the nurse include in a teaching plan that focuses on prevention of rheumatic fever?

a. Use corticosteroids to reduce inflammation
b. Provide antibiotic before dental work
c. Give penicillin to patients for treatment
d. Seek immediate treatment for throat infection

4 ميدترم 2024 (ترم أول 2023-2024)

One minor manifestation of rheumatic fever is:

a. Erythema marginatum
b. Polyarthritis and chorea
c. Increased sedimentation rate
d. Painless subcutaneous nodules

5 أسئلة سابقة

Symptoms of rheumatic fever typically appear how long after a streptococcal infection?

a. 1–2 days
b. 3–5 days
c. 2–3 weeks
d. 1 month

6 Which joints are most commonly affected by pain or swelling in rheumatic fever? أسئلة سابقة

a. Shoulders and hips
b. Knees, ankles, elbows and wrists
c. Fingers and toes
d. Spine and neck

7 What type of joint pain is often seen in rheumatic fever? أسئلة سابقة

a. Persistent pain in one joint
b. Pain that moves from joint to joint (migratory)
c. Pain relieved by rest
d. Pain without swelling or redness

8 Which is NOT a typical manifestation of rheumatic fever? أسئلة سابقة

a. Fever
b. Joint swelling
c. Skin rash
d. Headache

9 Under the 2015 Modified Jones Criteria, which symptom most likely indicates rheumatic fever? أسئلة سابقة

a. Severe headache
b. Hot or tender joints
c. Abdominal pain
d. Nausea and vomiting

10 Which body systems are typically affected in rheumatic fever? أسئلة سابقة

a. Respiratory, digestive and immune
b. Musculoskeletal, cardiovascular and nervous
c. Digestive, endocrine and respiratory
d. Cardiovascular, digestive and reproductive

11 Under the 2015 Modified Jones Criteria, which is a common sign? أسئلة سابقة

a. Skin lesions
b. Fever and joint pain
c. Gastrointestinal distress
d. Shortness of breath

12 Which is the characteristic skin manifestation of rheumatic fever? أسئلة سابقة

a. Erythema nodosum
b. Erythema marginatum
c. Psoriasis
d. Vitiligo

13 What is the primary treatment of rheumatic fever (eradicating the streptococcus)? أسئلة سابقة

a. Corticosteroids
b. Aspirin
c. Penicillin
d. Sulfadiazine

14 Which is NOT a sign or symptom of rheumatic fever? أسئلة سابقة

a. Joint pain
b. Fatigue
c. Chest pain
d. Nausea

15 One minor manifestation for rheumatic fever is أسئلة سابقة

a. Erythema marginatum
b. Poly arthritis and chorea
c. Increased sedimentation rate
d. Painless subcutaneous nodules

16 A nurse is preparing child for discharge after diagnosed with rheumatic fever that is the following instruction

أسئلة سابقة

a. Only give penicillin for dental work
b. Give aspirin for management chorea
c. Give penicillin until 21 years old
d. Give penicillin for one month total

17 Which of the following nursing diagnosis is most appropriate for child with rheumatic fever?

أسئلة سابقة

a. High risk for injury
b. Imbalance munition more than requirement
c. Delayed growth and development
d. Impaired gas exchange

18 In case of rheumatic heart fever with Myocarditis, the chest x-ray reveals normal heart size

أسئلة سابقة

a. True
b. False

19 Rheumatic fever is inflammation of the joints, heart, skin, and nervous system that occurs in children as a complication of untreated streptococcal infection of the throat. While most people with rheumatic fever recover, which part of the body may be permanently damaged in a small percentage of people?

أسئلة سابقة

a. Digestive tract
b. Ears
c. Heart
d. Lungs

20 Which of the following is usually the first symptom of rheumatic fever?

أسئلة سابقة

a. Chest pain or palpitations
b. Jerky, uncontrollable movements
c. Joint pain and fever
d. Rash

21 Penicillin is safely given to patients by the IV route. (True / False)

أسئلة سابقة

a. True
b. False

22 Each of the following drugs is used to treat rheumatic fever in children EXCEPT:

أسئلة سابقة

a. Acetaminophen
b. Antibiotics
c. Anti-inflammatory drugs
d. Corticosteroids

23 Patient with polyarthritis and carditis without evident of CHF (congestive heart failure) is treated with

أسئلة سابقة

a. IV salicylates
b. Oral salicylates
c. Corticosteroid
d. Both B and C

24 Which symptom presented is NOT a sign of rheumatic fever?

أسئلة سابقة

a. Truncal rash
b. Joint tenderness
c. Changes in vision
d. Nausea

25 Rheumatic fever is caused by which of the following bacteria?

أسئلة سابقة

a. Pseudomonas
b. Staphylococcus
c. Streptococcus
d. Shingella

26 Which of the following factors in the patient's history and physical is LEAST relevant to the diagnosis of Rheumatic fever? أسئلة سابقة

a. Sore throat from 3 weeks previous
b. Living in the current conditions of Flint, MI
c. The mother's history of type-2 diabetes
d. A possible genetic factor from the father who's history is unknown

27 The provider suspects rheumatic fever and orders blood work. Which results indicate inflammation and cardiac involvement? (Select all that apply) أسئلة سابقة

a. Elevated C-reactive protein
b. Decreased erythrocyte sedimentation rate
c. Elevated WBC count
d. Decreased hemoglobin and hematocrit

28 According to the Jones criteria, which sets of findings would indicate a positive diagnosis of rheumatic fever? (Select all that apply) أسئلة سابقة

a. Carditis, fever, and an elevated WBC count
b. Positive strep throat culture, arthritis, chorea
c. Recent scarlet fever, carditis, fever, arthralgia
d. Elevated C-reactive protein, carditis, fever

29 Acute Rheumatic Fever (ARF) is primarily caused by which bacterial species? أسئلة سابقة

a. Staphylococcus aureus
b. Streptococcus pneumoniae
c. Streptococcus agalactiae
d. Streptococcus pyogenes (Group A Streptococcus)

30 Molecular mimicry in Acute Rheumatic Fever refers to: أسئلة سابقة

a. The ability of bacteria to mimic host cells.
b. The interaction between antibodies and antigens.
c. The cross-reactivity between bacterial antigens and host tissues.
d. The production of toxins by Streptococcus pyogenes.

31 Which of the following clinical manifestations is NOT a major criterion for diagnosing Acute Rheumatic Fever? أسئلة سابقة

a. Carditis
b. Chorea
c. Elevated C-reactive protein
d. Subcutaneous nodules

32 Patient with polyarthritis and carditis with evident cardiomegaly is treated with أسئلة سابقة

a. IV salicylates
b. Oral salicylates
c. Corticosteroid
d. Adrenaline

33 Which of the following in not considered among supportive therapies for patient with carditis

أسئلة سابقة

a. Digoxin
b. Diuretics
c. Oxygen
d. Increasing fluid and salt intake

34 Which clinical manifestation of Acute Rheumatic Fever involves rapid, jerky movements and behavioral changes?

أسئلة سابقة

a. Carditis
b. Arthritis
c. Chorea
d. Subcutaneous nodules

35 Which genetic marker can influence an individual's susceptibility to developing Acute Rheumatic Fever after a streptococcal infection?

أسئلة سابقة

a. HLA-B27
b. Rh factor
c. ABO blood type
d. Genetic markers associated with the immune response

36 Which of the following is a major clinical manifestation of Acute Rheumatic Fever that involves inflammation of the heart?

أسئلة سابقة

a. Chorea
b. Arthritis
c. Carditis
d. Subcutaneous nodules

37 Which of the following is NOT a potential complication of Acute Rheumatic Fever?

أسئلة سابقة

a. Rheumatic heart disease
b. Glomerulonephritis
c. Meningitis
d. Chorea

38 Which of the following is a minor criterion for diagnosing Acute Rheumatic Fever?

أسئلة سابقة

a. Carditis
b. Arthritis
c. Elevated erythrocyte count
d. Subcutaneous nodules

39 What is the gold standard for diagnosing Group A Streptococcus infection in the laboratory?

أسئلة سابقة

a. Rapid strep test
b. Gram staining
c. Throat culture
d. Molecular PCR test

40 Which of the following medications is NOT commonly used in the treatment of Acute Rheumatic Fever?

أسئلة سابقة

a. Penicillin
b. Nonsteroidal anti-inflammatory drugs (NSAIDs)
c. Corticosteroids
d. Antibiotics targeting Gram-negative bacteria

41 Rheumatic fever primarily affects which of the following body systems?

أسئلة سابقة

a. Digestive system
b. Musculoskeletal system
c. Respiratory system
d. Nervous system

42 Which of the following is the characteristic feature of rheumatic fever? أسئلة سابقة

a. Joint pain and swelling
b. Skin rash
c. Heart murmurs
d. All of the above

43 Which of the following age groups are at higher risk for rheumatic fever? Select the correct answer أسئلة سابقة

a. Adults 50 and above
b. Children ages 5-15
c. Infants
d. Women ages 20-40

44 The best way to prevent rheumatic fever is: أسئلة سابقة

a. Avoiding contact with infected individuals
b. Getting vaccinated
c. Early diagnosis and treatment of streptococcal infections
d. Washing hands frequently

45 Rheumatic fever can lead to long-term complications, primarily affecting the..... أسئلة سابقة

a. Heart
b. Kidneys
c. Liver
d. Lungs

46 The condition characterized by scarring and damage to the heart valves as a consequence of rheumatic fever is called..... أسئلة سابقة

a. Coronary artery disease
b. Rheumatic heart disease
c. Rheumatoid arthritis
d. Septicemia

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 Rheumatic fever is an autoimmune reaction that follows infection with: NCLEX

a. Staphylococcus aureus
b. Group A beta-hemolytic streptococcus
c. Pneumococcus
d. E. coli

N2 Select ALL MAJOR Jones criteria. SELECT ALL

a. Carditis
b. Polyarthritis
c. Chorea
d. Erythema marginatum
e. Subcutaneous nodules
f. Elevated ESR

N3 Secondary prophylaxis after rheumatic fever usually consists of: NCLEX

a. Oral aspirin for one month
b. Benzathine penicillin IM every 3–4 weeks for years
c. Corticosteroids for life
d. Antibiotics only during dental work

N4 **A child with RF has chorea. The priority nursing diagnosis is:** NCLEX

a. Impaired gas exchange
b. Risk for injury
c. Constipation
d. Excess fluid volume

N5 **Which finding indicates carditis in a child with rheumatic fever?** NCLEX

a. New heart murmur and tachycardia out of proportion to fever
b. Migratory joint pain
c. Ring-shaped rash
d. Painless nodules on extensor surfaces

N6 **The joint involvement in rheumatic fever is typically:** NCLEX

a. Small joints, symmetric and deforming
b. Migratory polyarthritis of large joints, non-deforming
c. Spinal only
d. Single joint with permanent damage

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Cause	Autoimmune reaction 2–3 weeks after group A beta-hemolytic strep pharyngitis.	**Molecular mimicry**	Cross-reactivity between streptococcal antigens and host tissues.
Major Jones criteria	Carditis, polyarthritis, chorea, erythema marginatum, subcutaneous nodules.	**Minor criteria**	Fever, arthralgia, raised ESR/CRP, prolonged PR interval.
Evidence of strep	Positive throat culture (gold standard) or raised ASO titer.	**Arthritis**	Migratory, large joints, non-deforming.
Chorea (Sydenham)	Involuntary jerky movements, emotional lability; risk for injury.	**Carditis**	Only manifestation causing permanent damage → rheumatic heart disease (mitral).
Treatment	Penicillin, salicylates for arthritis, steroids for carditis with cardiomegaly/CHF, bed rest.	**Secondary prophylaxis**	Benzathine penicillin IM every 3–4 weeks (often until 21 years).
Primary prevention	Prompt treatment of strep throat.	**Age group**	Children 5–15 years.

Answer Key

الإجابات

1. a — High risk for injury

2. c — Give penicillin until 21 years old

3. d — Seek immediate treatment for throat infection
ورقة الطالب معلّم عليها corticosteroid الوقاية؛ وده علاج مش وقاية الأولية = علاج التهاب الحلق البكتيري بسرعة.

4. c — Increased sedimentation rate

5. c — 2–3 weeks
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

6. b — Knees, ankles, elbows and wrists
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

7. b — Pain that moves from joint to joint (migratory)
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

8. d — Headache
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

9. b — Hot or tender joints
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

10. b — Musculoskeletal, cardiovascular and nervous
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

11. b — Fever and joint pain
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

12. b — Erythema marginatum
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

13. c — Penicillin
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

14. d — Nausea
(المصدر مفيهوش إجابة رسمية) إجابة مقترحة.

15. c — Increased sedimentation rate

16. c — Give penicillin until 21 years old

17. a — High risk for injury

18. b — False

19. c — Heart

20. c — Joint pain and fever

21. b — False

22. a — Acetaminophen

23. b — Oral salicylates

24. c — Changes in vision

25. c — Streptococcus

26. c — The mother's history of type-2 diabetes

27. a, c — Elevated C-reactive protein; Elevated WBC count

28. b, c — Positive strep throat culture, arthritis, chorea; Recent scarlet fever, carditis, fever, arthralgia

29. d — Streptococcus pyogenes (Group A Streptococcus)

30. c — The cross-reactivity between bacterial antigens and host tissues.

31. c — Elevated C-reactive protein

32. c — Corticosteroid

33. d — Increasing fluid and salt intake

34. c — Chorea

35. d — Genetic markers associated with the immune response

36. c — Carditis

37. c — Meningitis

38. b — Arthritis
إجابة البنك (Arthritis) الـ غير دقيقة: polyarthritis معيار major، والمعيار الـ minor هو arthralgia. الأقرب هي b. لو الاختيارات دي جت،

39. c — Throat culture

40. d — Antibiotics targeting Gram-negative bacteria

41. b — Musculoskeletal system
إجابة البنك: musculoskeletal (المفاصل أشيع عرض). لكن أخطر وأدوم تأثير على القلب.

42. d — All of the above

43. b — Children ages 5-15

44. c — Early diagnosis and treatment of streptococcal infections

45. a — Heart

46. b — Rheumatic heart disease

N1. b — Group A beta-hemolytic streptococcus
Molecular mimicry between streptococcal antigens and host tissues (heart, joints, brain, skin) causes RF.

N2. a, b, c, d, e — Carditis; Polyarthritis; Chorea; Erythema marginatum; Subcutaneous nodules
The 5 major criteria are carditis, polyarthritis, chorea, erythema marginatum and subcutaneous nodules; elevated ESR/CRP is minor.

N3. b — Benzathine penicillin IM every 3–4 weeks for years
Long-acting penicillin prevents recurrence; duration depends on carditis (often until age 21 or longer).

N4. b — Risk for injury
Involuntary jerky movements put the child at risk of falls and injury — provide a safe, quiet environment.

N5. a — New heart murmur and tachycardia out of proportion to fever
Carditis presents with new murmurs, tachycardia, cardiomegaly or pericardial rub; it is the only manifestation that can cause permanent damage.

N6. b — Migratory polyarthritis of large joints, non-deforming
RF arthritis migrates between large joints (knees, ankles, elbows, wrists) and heals without deformity.